Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 07-Apr-2026

---

1. Administrative & Study Identification

Official Full Study Title: A Phase III, Randomized, Double-blind Study of Tiragolumab Plus Atezolizumab Compared With Placebo Plus Atezolizumab in Participants With Completely Resected Stage IIB, IIIA, or Select IIIB, PD-L1 Positive, Non-small Cell Lung Cancer Who Have Received Adjuvant Platinum-based Chemotherapy **Study Title:** A Study of Tiragolumab Plus Atezolizumab Compared With Placebo Plus Atezolizumab in Participants With Completely Resected Non-small Cell Lung Cancer Who Have Received Adjuvant Platinum-based Chemotherapy **Short Title/Acronym:** SKYSCRAPER-15 **Protocol Number:** GO45006 **NCT Number:** NCT06267001 **Posting ID:** 371427935541525776 **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Sponsor/Company Name:** Hoffmann-La Roche / Roche **Investigational Product:** Tiragolumab + Atezolizumab **Phase of Development:** Phase III (PHASE3) **Phase Requirement:** Trial must be in PHASE3 (Phase ID: PHASE3) **Source Level:** 5 **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

---

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To evaluate the safety and tolerability (specifically the percentage of participants with Adverse Events) of tiragolumab plus atezolizumab compared with placebo plus atezolizumab administered to participants with non-small cell lung cancer (NSCLC) following resection and adjuvant chemotherapy. *(Note: With Protocol Amendment 2, enrollment will be stopped. The primary objective of the study has been changed to a safety objective and no other analysis will be conducted).* * **Secondary Objectives:** To evaluate progression-free survival (PFS), disease-free survival (DFS), and overall survival (OS), as well as to monitor ongoing immune-related adverse events.

2.2 Background & Rationale Non-small cell lung cancer (NSCLC) is the most common type of lung cancer. In early-stage NSCLC, surgery and chemotherapy are the standard of care, but the risk of cancer recurring remains significant. Atezolizumab blocks the PD-L1 protein to help the immune system fight cancer cells. Tiragolumab is a novel anti-TIGIT immunotherapy designed to further boost anti-cancer immune activity when given in combination with atezolizumab. This study aims to determine if this combination is safe and effectively reduces the risk of recurrence in early-stage NSCLC.

---

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Placebo-controlled * **Blinding:** Double-blind * **Randomization:** Randomized (1:1)

3.2 Planned Enrollment & Duration * **Targeted Enrollment (N):** 56 participants *(Enrollment stopped early per Protocol Amendment 2)* * **Number of Sites:** Global multi-center trial (spanning multiple countries including Australia, Argentina, Belgium, etc.) * **Estimated Study Start:** 21-Mar-2024 * **Estimated Primary Completion:** 16-Dec-2025

---

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 2. Histological or cytological diagnosis of Stage IIB, IIIA, and select IIIB NSCLC of either non-squamous or squamous histology. 3. Tumor cell PD-L1 expression at $\ge 1\%$. 4. Participants must have had complete surgical resection of NSCLC. 5. Participants must have received between one to four cycles of adjuvant histology-based platinum doublet chemotherapy. 6. Participants must have recovered adequately from surgery and from adjuvant chemotherapy. 7. Adequate hematologic and end-organ function.

4.2 Exclusion Criteria 1. Any history of prior NSCLC within the last 5 years. 2. Any evidence of residual disease or disease recurrence following surgical resection of NSCLC, or during or following adjuvant chemotherapy. 3. NSCLC known to have a mutation in the epidermal growth factor receptor (EGFR) gene or an anaplastic lymphoma kinase (ALK) fusion oncogene.

---

5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** Tiragolumab plus atezolizumab OR placebo plus atezolizumab administered via co-infusion once every month. *(Atezolizumab Drug Name / Preferred Name: atezolizumab; Trade Name: Tecentriq; ATC Code: L01FF05; EMA Product Information: [EMA URL])* * **Route of Administration:** Intravenous (IV). * **Duration of Treatment:** Up to approximately 1 year (13 treatment cycles) unless the cancer recurs sooner or unacceptable toxicity occurs.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive care and concomitant medications for co-morbidities. * **Prohibited:** Any other investigational agents, concurrent systemic anti-cancer therapies, or live vaccines during the treatment period.

---

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor tissue testing (PD-L1/EGFR/ALK), ECOG, Imaging | | **Baseline** | Day 0 | Randomization, First IV Co-infusion | | **Interim** | Monthly | IV Infusions (Cycles 2-13), Safety Labs, AE Monitoring, Efficacy Scans | | **End of Study** | Post-Tx | Final assessment 1 month after the last dose, then survival follow-up every 3 months |

---

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Definition:** Percentage of participants experiencing Adverse Events (AEs) and Serious Adverse Events (SAEs) as a measure of safety and tolerability. * **Measurement Method:** Clinical investigator assessment, laboratory test grading, and continuous AE monitoring throughout the trial.

7.2 Secondary & Exploratory Endpoints * Disease-Free Survival (DFS) (Time from randomization to disease recurrence or death). * Overall Survival (OS) (Time from randomization to death from any cause).

---

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Standard collection via monthly onsite visits and laboratory investigations, specifically looking for immune-related adverse events (irAEs).
- **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness.
- **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (iDMC) to oversee unblinded safety data periodically.

---

9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Safety Set (all participants who received at least one dose of the investigational product).
- **Sample Size Justification:** $N=56$; the original power calculation for efficacy was superseded by Protocol Amendment 2, terminating enrollment and shifting the primary analysis strictly to safety outcomes.
- **Handling of Missing Data:** Standard imputation for missing AE start/stop dates; right-censoring for survival/DFS data at the last known date the patient was event-free.

---

10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
- **Monitoring Plan:** Regular onsite and remote monitoring visits by the sponsor to ensure protocol adherence and safety.
- **Audit Trail:** Comprehensive eCRF locking mechanisms and source data verification (SDV) to guarantee data integrity.

---

11. Study Identifiers & Governance

- **Primary ID:** GO45006
- **Registry Number:** NCT06267001
- **Posting ID:** 371427935541525776
- **Sponsor/Lead Organization:** Hoffmann-La Roche / Roche
- **Study Short Title:** SKYSCRAPER-15
- **General Study Title:** A Study of Tiragolumab Plus Atezolizumab Compared With Placebo Plus Atezolizumab in Participants With Completely Resected Non-small Cell Lung Cancer Who Have Received Adjuvant Platinum-based Chemotherapy

---

12. Clinical Framework, Ontology & Classifications

- **Primary Condition & Disease Area:** Non-Small Cell Lung Cancer (NSCLC)
- **Condition Name:** Non-Small Cell Lung Cancer (NSCLC)
- **Preferred UMLS Name:** Non-small cell lung carcinoma
- **MeSH Code:** D002275
- **SNOMED ID:** 254635002
- **SNOMED Hierarchy:** 254637007|Malignant neoplasm of lung|Malignant neoplasm of lung (disorder)
- **Diagnostic Concept Preferred Name:** Non-Small Cell Lung Cancer (NSCLC) Diagnostic Reagents
- **Semantic Type:** Indicator, Reagent, or Diagnostic Aid
- **Diagnostic Reagent Definition:** Molecular assay reagents intended for use in identifying translocations (i.e., exchanges of chromosomal material) involving the anaplastic lymphoma kinase (ALK) gene at chromosomal location 2p23. Translocations involving ALK may result in the generation of fusion genes, which have been implicated in the pathophysiology of multiple cancers (e.g., anaplastic large cell lymphoma, non-small cell lung cancer, renal cell carcinoma). Multiple fusion partners for ALK have been identified in human cancers, including EML4, KIF5B, NPM1, SQSTM1, and TPM3. These reagents are intended to identify translocations involving the ALK gene and any gene fusion partner.
- **Diagnosis Threshold:** Completely resected Stage IIB, IIIA, or select IIIB, PD-L1 $\ge 1\%$

- **Medical Methodology:** Adjuvant Immunotherapy
- **Optimization Target:** Prevention of disease recurrence and evaluation of combination safety profile

---

13. Study Procedures & Methodology

- **Management Pathway:** Standard adjuvant clinical pathway for early-stage resected NSCLC
- **Education Protocol (HCP):** Identification and management of immune-related adverse events (irAEs)
- **Education Protocol (Patient):** Counseling on side effects of IV co-infusions and reporting timelines
- **Quality Audit Mechanism:** Centralized safety monitoring and source data verification

---

14. Observation & Follow-Up Schedule

- **Total Duration:** 1 year of active treatment + long-term follow-up (up to study end)
- **Onsite Visit Cadence:** Monthly (every cycle) for 13 cycles
- **Remote Monitoring Frequency:** Every 3 months (Q12W) during post-treatment follow-up
- **Checklist Review Interval:** Prior to every IV infusion visit

---

15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]